The typical manifestation of intestinal amoebiasis is amoebic dysentery. The symptoms are often quite mild and can include loose feaces, stomach pain and stomach cramping. In acute amoebic dysentery, the symptoms include abdominal pain, bloody stool, fever, tenderness, rectal tenesmus and hepatomegaly (enlargement of liver). People affected may develop anemia due to loss of blood. On clinical and laboratory ground, amoebic dysentery should be differentiated from bacillary dysentery. A Table 8.1 shows the difference between the stools of amoebic and bacillary dysentery.

Extra intestional amoebiasis

1. **Hepatic amoebiasis:** This is the most common form of extra intestinal invasive amoebiasis. Liver abscess may be multiple or more often solitary, usually located in the upper right lobe of the liver (Figure 8.4). Amoebic liver abscess (ALA) contains an odour less and thick chocolate brown pus called anchovy sauce pus. ALA is associated with an abrupt onset of high fever, right upper abdominal pain and tenderness. Anorexia (loss of appetite for food), nausea (the sensation to vomit), vomiting, fatigue (extreme tiredness) and weight loss are also frequent.

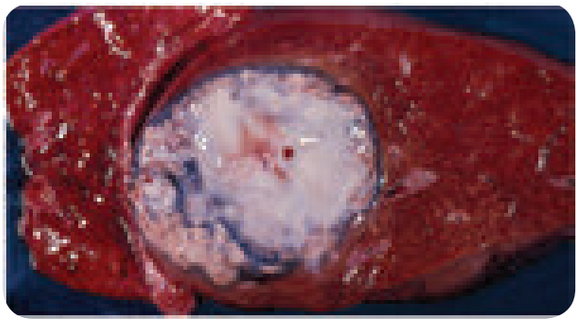


Figure 8.4: Amoebic liver abscess

2. **Pulmonary Amoebiasis:** It is very rare, but this may occur when direct hematogenous spread from the colon. The patient presents with severe chest

Table 8.1: Difference between the stools of amoebic and bacillary dysentery

	Amoebic dysentery	Bacillary Dysentery
Macroscopic observation		
Number	6–8	Over 10 motions a day
Amount	Relatively large	Small
Odour	Offensive	Odorless
Colour	Dark red	Bright red
Nature	Blood and mucus mixed with faeces	Blood and mucus no faeces
Microscopic observation		
RBC	In clumps, reddish – yellow in colour	RBC in rouleaux, bright red in colour
Pus cells	Scanty	Numerous
Parasite	Trophozoites of *E. histolytica*	Nil
Charcot – Leyden crystals	Present	Nil

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